

ST. JUDE REGIONAL MEDICAL CENTER - INPATIENT CARDIOLOGY

CLINICAL DISCHARGE SUMMARY & DOSSIER REPORT

Patient Name: Jonathan Doe	MRN: MRN-2026-9812	DOB: 1964-04-12 (62 yrs)
Admission Date: 2026-09-28	Discharge Date: 2026-10-02	Attending: Dr. Sarah Jenkins, MD (Cardiology)

REASON FOR ADMISSION & CLINICAL COURSE

Jonathan Doe is a 62-year-old male with a history of hypertension and type 2 diabetes mellitus who presented to the Emergency Department experiencing acute substernal chest pressure radiating to the left jaw and diaphoresis on exertion. Symptoms began approximately 3 hours prior to presentation. In the ED, the patient was hemodynamically stable. An urgent CTA showed no acute pulmonary embolism or aortic dissection. Initial troponin elevation confirmed an Acute Non-ST Elevation Myocardial Infarction (NSTEMI). The patient was started on dual antiplatelet therapy and anticoagulation and admitted to the Cardiac Care Unit (CCU).

DIAGNOSTIC INVESTIGATIONS & LABORATORY RESULTS

Investigation	Result Value	Reference Range	Clinical Interpretation
High-Sensitivity Troponin T	142 ng/L	< 14 ng/L	Markedly Elevated (Myocardial Necrosis)
12-Lead Electrocardiogram (ECG)	Sinus rhythm, 74 bpm	Normal	1.5mm ST-segment depression leads V4-V6
Transthoracic Echocardiogram (TTE)	LVEF 55%	50% - 70%	Hypokinesis of inferolateral wall
Serum Creatinine	1.0 mg/dL	0.7 - 1.3 mg/dL	Normal renal function baseline
Fasting Serum Glucose	138 mg/dL	70 - 99 mg/dL	Mildly elevated, consistent with T2DM
Serum Potassium	4.2 mEq/L	3.5 - 5.0 mEq/L	Normal electrolyte homeostasis

INVASIVE PROCEDURES PERFORMED

Coronary Angiography and Percutaneous Coronary Intervention (PCI): Performed on hospital day 2 via right radial approach. Angiography demonstrated an 80% eccentric stenosis in the mid-left anterior descending (LAD) artery. Successful pre-dilation and deployment of a 3.0 x 18 mm Drug-Eluting Stent (DES) with 0% residual stenosis and TIMI-3 flow restored. No procedural complications.

DISCHARGE DIAGNOSES

Confirmed Diagnoses:

- Acute Non-ST Elevation Myocardial Infarction (NSTEMI) - Post-PCI to LAD
- Essential Hypertension (HTN) - Controlled
- Type 2 Diabetes Mellitus (T2DM) - Chronic, non-insulin dependent

Rule-Out / Negated Conditions:

- Ruled out Acute Pulmonary Embolism (PE) - CT pulmonary angiography negative
- Ruled out Acute Aortic Dissection - Normal mediastinum and aortic lumen

DISCHARGE MEDICATIONS REGIMEN

Medication Name	Dose	Route	Frequency	Indication
Atorvastatin	80 mg	Oral	Once daily at bedtime	High-intensity statin / Atherosclerosis
Metoprolol Tartrate	25 mg	Oral	Twice daily with meals	Beta-blocker / Post-MI cardioprotection
Aspirin (Enteric Coated)	81 mg	Oral	Once daily in morning	Antiplatelet therapy
Metformin HCl	500 mg	Oral	Twice daily with meals	Type 2 Diabetes Mellitus control
Lisinopril	10 mg	Oral	Once daily in morning	ACE inhibitor / Hypertension & LV protection

FOLLOW-UP INSTRUCTIONS & OUTSTANDING ACTIONS

- 1. **Cardiology Clinic Follow-up:** Appointment with Dr. Sarah Jenkins in 2 weeks for clinical post-PCI review and wound check.
- 2. **Laboratory Re-evaluation:** Fasting Lipid Panel and Comprehensive Metabolic Panel (CMP) repeat in 6 weeks.
- 3. **Cardiac Rehabilitation:** Enrollment in Phase II Cardiac Rehab Program starting within 3 weeks.

Electronically Verified & Signed: **Dr. Sarah Jenkins, MD, FACC** - License #MD-883921
St. Jude Regional Medical Center Cardiology Service